

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Testogel, Tostran, Sustanon and Nebido for the treatment of Low Testosterone

Summary of NICE / NICE CKS Guidelines for Low Testosterone (Male Hypogonadism)

Overview

- **Low testosterone** (hypogonadism) refers to **clinical symptoms** associated with **low serum testosterone levels**.
- Can be due to:
 - **Primary hypogonadism** (testicular failure)
 - **Secondary hypogonadism** (pituitary/hypothalamic dysfunction)
 - **Functional hypogonadism** (e.g. due to obesity, diabetes, chronic illness)

Symptoms of Low Testosterone

- Reduced libido or erectile dysfunction
- Fatigue or low energy
- Low mood or depression
- Reduced muscle mass or strength
- Increased body fat
- Infertility
- Osteoporosis or low bone density

Symptoms are **non-specific**, and a **careful history and clinical judgement** are essential.

Assessment

Indications for Testing

- Symptoms suggestive of hypogonadism
- Delayed or incomplete puberty
- Osteoporosis or low bone density in men
- Infertility

Investigations

- **Early morning total testosterone** (7–11 am) — fasting if possible
 - If low (<12 nmol/L), **repeat** to confirm
 - Consider **sex hormone-binding globulin (SHBG)** and **free testosterone** if total testosterone borderline (12–15 nmol/L)
 - **LH and FSH** to differentiate:
 - **High LH/FSH** = primary hypogonadism
 - **Low or normal LH/FSH** = secondary hypogonadism
 - Additional tests:
 - **Prolactin, pituitary function tests, MRI pituitary** if secondary cause suspected
 - **Karyotyping** if primary hypogonadism with small testes (e.g. Klinefelter's)
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Management

Lifestyle and Treat Underlying Cause

- Weight loss, reduce alcohol intake
- Optimise management of comorbidities (e.g. diabetes, sleep apnoea)
- Address medication causes (e.g. opioids, steroids)

Testosterone Replacement Therapy (TRT)

- Consider if:
 - **Confirmed low testosterone AND**
 - **Consistent symptoms of hypogonadism**
- Not recommended for **age-related testosterone decline** alone without symptoms

Formulations:

- **Transdermal gels/patches** (e.g. testosterone gel 50 mg daily)
- **Long-acting intramuscular injections** (e.g. testosterone undecanoate every 10–14 weeks)
- **Oral formulations** (less preferred)

Choice depends on patient preference, availability, and tolerability.

Monitoring on TRT

- **Testosterone level** (aim for mid-normal range):
 - 2–3 months after starting
 - Then every 6–12 months
- Monitor:
 - **PSA** (especially in men >40)
 - **FBC** (check for polycythaemia)

- **LFTs, lipids, weight/BMI**
 - **Bone density** if osteoporosis present
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Contraindications to TRT

- **Prostate or breast cancer**
 - **Elevated PSA without investigation**
 - **Severe untreated sleep apnoea**
 - **Polycythaemia (haematocrit >0.54)**
 - **Uncontrolled heart failure**
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Referral to Endocrinology

- Diagnostic uncertainty
- Suspected pituitary disease
- Fertility concerns
- Contraindications to TRT or complex comorbidities

Testing and monitoring regimen (all testosterone products)

Initial diagnosis: two early-morning fasting total testosterone measurements, taken at least four weeks apart, before treatment is offered.

Baseline panel (before first supply): total testosterone (early morning, fasting); free or calculated free testosterone; SHBG; LH and FSH; prolactin; full blood count and haematocrit; U&Es/eGFR; liver function tests; lipid profile; HbA1c or fasting glucose; PSA in men aged 40 and over.

Follow-up panel (6 to 12 weeks after initiation): testosterone level; full blood count and haematocrit; liver function tests; PSA where required (aged 40 and over).

Ongoing monitoring: every 3 to 6 months in the first year, then annually once stable — testosterone level; full blood count and haematocrit; liver function tests; lipid profile; PSA where required (aged 40 and over).

PSA is not routinely required for men under 40; it is offered as a separate add-on for men aged 40 and over.

Supply intervals: 28-day supplies for topical preparations aligned to review appointments (testosterone is a Schedule 4 controlled drug; 30 days is the good-practice maximum). Depot injection supply follows the product dosing interval.

Patient Group Direction

for the supply/administration of

Testogel (testosterone gel)

for the treatment of

Low Testosterone

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Testogel is indicated for testosterone replacement therapy in adult men with confirmed primary or secondary hypogonadism associated with clinical symptoms and biochemical evidence of low serum testosterone.
Inclusion criteria	- Adult males aged 18 years and over. - Confirmed diagnosis of hypogonadism (low serum testosterone with symptoms). - Informed consent obtained. - No contraindications to testosterone therapy.
Exclusion criteria	- Known or suspected carcinoma of the breast or prostate. - History of liver tumours or severe hepatic insufficiency. - Polycythaemia (haematocrit >54%). - Untreated obstructive sleep apnoea. - Pregnancy or breastfeeding (contraindicated for female partners if risk of transference). Severe cardiac or renal disease.
Cautions	- Regularly monitor testosterone levels, haematocrit, PSA, and clinical response. - Avoid skin contact with others post-application; advise hand washing and covering site with clothing. - Discontinue if no clinical benefit or if significant adverse effects occur. - Counsel on application technique, safe handling, and disposal. Limited therapeutic experience in patients over 65 years of age — use with caution and review ongoing clinical benefit regularly.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Testogel
Legal category	POM
Storage	Store below 25°C. Do not refrigerate or freeze.
Route/method of administration	Topical application to clean, dry, intact skin (shoulders, upper arms, or abdomen).
Dose and frequency	Recommended starting dose is 50 mg of testosterone (one sachet or 1-2 pump actuation) once daily, preferably in the morning.
Quantity to be administered and/or supplied	28-day supply based on 50–100 mg daily dose; adjusted according to serum testosterone levels.

Maximum or minimum treatment period	Reassess at 3–6 months and then annually to confirm ongoing clinical benefit.
Adverse effects	Skin irritation, acne, increased red blood cell count, mood changes, prostate symptoms. Rare: polycythaemia, liver effects. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
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- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
13/07/26		12/07/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		13/07/26
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025
002	Reviewed and authorised for the GRH platform; signed by Medical Director	13th July 2026
003	Added shared testing and monitoring regimen (BSSM-aligned panels incl. U&Es baseline; PSA 40+ as separate add-on); severe cardiac or renal disease added to exclusions; over-65 caution added; supply interval guidance. Agreed with C. Pilkington and PPH clinical review.	29th July 2026

Patient Group Direction

for the supply/administration of

Tostran (testosterone gel)

for the treatment of

Low Testosterone

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Tostran is indicated for testosterone replacement therapy in adult men with confirmed primary or secondary hypogonadism with clinical symptoms and laboratory confirmation of testosterone deficiency.
Inclusion criteria	- Adult males aged 18 years and over. - Confirmed diagnosis of hypogonadism (low serum testosterone with symptoms). - Informed consent obtained. - No contraindications to testosterone therapy.
Exclusion criteria	- Known or suspected carcinoma of the breast or prostate. - History of liver tumours or severe hepatic impairment. - Polycythaemia (haematocrit >54%). - Untreated obstructive sleep apnoea. - Risk of testosterone transference to others (household contact not manageable). Severe cardiac or renal disease.
Cautions	- Monitor testosterone levels, haematocrit, PSA, and symptoms regularly. - Counsel on application technique, hygiene, and avoidance of transference. - Rotate application sites and avoid broken skin. - Reassess if inadequate clinical response or if adverse effects occur. Limited therapeutic experience in patients over 65 years of age — use with caution and review ongoing clinical benefit regularly.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Tostran gel
Legal category	POM
Storage	Store below 25°C. Do not refrigerate or freeze. Keep container tightly closed.
Route/method of administration	Topical application to clean, dry, intact skin on the abdomen or inner thighs, preferably in the morning.
Dose and frequency	Recommended starting dose is 3 g of gel (60 mg testosterone) once daily, adjusted based on clinical response and serum testosterone levels.
Quantity to be administered and/or supplied	One 60 g canister (approximately 30 doses), issued monthly or as appropriate to prescribed dose.

Maximum or minimum treatment period	Reassess at 3–6 months and then annually to confirm continued need and benefit.
Adverse effects	Application site reactions, acne, increased red blood cell count, mood changes, prostate enlargement. Rare: polycythaemia, liver effects. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

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Patient Group Direction

for the supply/administration of

Sustanon (testosterone injection)

for the treatment of

Low Testosterone

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training with Get Real Health in order to use this PGD. This training will be documented and signed off by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Sustanon is indicated for testosterone replacement therapy in adult men with confirmed primary or secondary hypogonadism with clinical symptoms and biochemical evidence of testosterone deficiency.
Inclusion criteria	- Adult males aged 18 years and over. - Confirmed diagnosis of hypogonadism with low serum testosterone. - Informed consent obtained. - No contraindications to testosterone therapy.
Exclusion criteria	- Known or suspected carcinoma of the prostate or breast. - Polycythaemia (haematocrit >54%). - Severe cardiac, hepatic or renal impairment. - Untreated obstructive sleep apnoea. - Known hypersensitivity to any components of the injection. Severe cardiac or renal disease.
Cautions	- Monitor testosterone levels, haematocrit, PSA, lipids, and liver function regularly. - Educate on potential adverse effects and long-term monitoring. - Ensure injections are given intramuscularly and not intravenously. - Reassess benefit/risk balance periodically during long-term therapy. Limited therapeutic experience in patients over 65 years of age — use with caution and review ongoing clinical benefit regularly.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Sustanon injection
Legal category	POM
Storage	Store below 30°C. Do not refrigerate or freeze. Protect from light.
Route/method of administration	Intramuscular injection into the gluteal muscle by a trained healthcare professional.
Dose and frequency	Typically 1 mL (250 mg) every 2-3 weeks. Adjust based on serum testosterone levels and clinical response.
Quantity to be administered and/or supplied	One ampoule per injection; sufficient for 2-3 weeks of treatment.
Maximum or minimum treatment period	Reassess at 3–6 months and then annually to confirm continued need and effectiveness.
Adverse effects	Injection site pain, acne, mood changes, increased red blood cell count,

	fluid retention, prostate effects. Rare: liver dysfunction. Refer to SmPC.
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

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Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

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002	Reviewed and authorised for the GRH platform; signed by Medical Director	13th July 2026
003	Added shared testing and monitoring regimen (BSSM-aligned panels incl. U&Es baseline; PSA 40+ as separate add-on); severe cardiac or renal disease added to exclusions; over-65 caution added; supply interval guidance. Agreed with C. Pilkington and PPH clinical review.	29th July 2026

Patient Group Direction

for the supply/administration of

Nebido (testosterone undecanoate injection)

for the treatment of

Low Testosterone

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Nebido is indicated for testosterone replacement therapy in adult men with hypogonadism associated with clinical symptoms and confirmed by biochemical tests demonstrating testosterone deficiency.
Inclusion criteria	- Adult males aged 18 years and over. - Confirmed diagnosis of testosterone deficiency (hypogonadism). - Informed consent obtained. - No contraindications to testosterone therapy.
Exclusion criteria	- Known or suspected prostate or breast carcinoma. - Severe cardiac, renal or hepatic disease. - Polycythaemia (haematocrit >54%). - Untreated obstructive sleep apnoea. - Hypersensitivity to testosterone undecanoate or formulation ingredients. Severe cardiac or renal disease.
Cautions	- Monitor serum testosterone, haematocrit, liver function, PSA, and clinical response routinely. - Counsel on long-term therapy, expected onset and duration of effects. - Ensure deep intramuscular injection; avoid intravenous use. - Monitor for symptoms of excessive androgen exposure or adverse effects. Limited therapeutic experience in patients over 65 years of age — use with caution and review ongoing clinical benefit regularly.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Nebido injection
Legal category	POM
Storage	Store below 30°C. Do not refrigerate or freeze. Protect from light.
Route/method of administration	Intramuscular injection into the gluteal muscle by a trained healthcare professional, administered slowly over 2 minutes.
Dose and frequency	1000 mg (4 mL) every 10–14 weeks following an initial interval of 6 weeks after the first dose.
Quantity to be administered and/or supplied	One vial per injection, administered every 10–14 weeks.
Maximum or minimum treatment period	Reassess treatment at 3–6 months and annually thereafter based on

	testosterone levels and clinical response.
Adverse effects	Injection site pain, acne, increased haematocrit, weight gain, mood changes, libido changes. Rare: polycythaemia, hepatic dysfunction. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

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